

Droperidol: Drug information - UpToDate

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For additional information see "Droperidol: Patient drug information" and "Droperidol: Pediatric drug information"

For abbreviations, symbols, and age group definitions show table

ALERT: US Boxed Warning

Arrhythmias:

Cases of QT prolongation and/or torsade de pointes have been reported in patients receiving droperidol at doses at or below recommended doses. Some cases have occurred in patients with no known risk factors for QT prolongation, and some cases have been fatal.

Due to its potential for serious proarrhythmic effects and death, reserve droperidol for use in the treatment of patients who fail to show an acceptable response to other adequate treatments, either because of insufficient effectiveness or the inability to achieve an effective dose due to intolerable adverse effects from those drugs.

Cases of QT prolongation and serious arrhythmias (eg, torsade de pointes) have been reported in patients treated with droperidol. Based on these reports, all patients should undergo a 12-lead ECG prior to administration of droperidol to determine if a prolonged QT interval (ie, QTc greater than 440 msec for males or 450 msec for females) is present. If there is a prolonged QT interval, do not administer droperidol. For patients in whom the potential benefit of droperidol treatment is felt to outweigh the risks of potentially serious arrhythmias, perform ECG monitoring prior to treatment and continue for 2 to 3 hours after completing treatment to monitor for arrhythmias.

Droperidol is contraindicated in patients with known or suspected QT prolongation, including patients with congenital long QT syndrome.

Administer droperidol with extreme caution to patients who may be at risk for development of prolonged QT syndrome (eg, congestive heart failure, bradycardia, use of a diuretic, cardiac hypertrophy, hypokalemia, hypomagnesemia, or administration of other drugs known to increase the QT interval). Other risk factors may include age greater than 65 years, alcohol abuse, and use of agents such as benzodiazepines, volatile anesthetics, and IV opiates. Initiate droperidol at a low dose and adjust upward, with caution, as needed to achieve the desired effect.

Pharmacologic Category

- Antiemetic;
- First Generation (Typical) Antipsychotic

Dosing: Adult

Note: Monitor vital signs and ECG at baseline and routinely.

Agitation, acute

Agitation, acute (undifferentiated) (off-label use):

IM: Initial: 5 to 10 mg, as monotherapy or in combination with a benzodiazepine; wait at least 10 to 30 minutes before providing additional medications for agitation if needed (Ref).

IV: Initial: 2.5 to 10 mg, as monotherapy or in combination with a benzodiazepine; may repeat droperidol dose every 5 minutes until sedation achieved; maximum dose 20 mg per episode; median required dose was 10 mg in one study (Ref).

Cannabinoid hyperemesis syndrome

Cannabinoid hyperemesis syndrome (off-label use):

IV: 0.625 to 2.5 mg single dose (Ref).

Postoperative nausea and vomiting

Postoperative nausea and vomiting (PONV):

IV: 0.625 to 1.25 mg administered at the end of procedure (Ref).

Manufacturer labeling: Dosing in the prescribing information may not reflect current clinical practice.

IM/IV: Maximum initial dose: 2.5 mg; additional doses of 1.25 mg may be administered with caution to achieve desired effect.

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling; use with caution.

Dosing: Liver Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling; use with caution.

Dosing: Older Adult

Note: Avoid for behavioral problems associated with dementia or delirium unless alternative non-pharmacologic therapies have failed and patient may harm self or others. If used, consider de-prescribing attempts to assess continued need and/or lowest effective dose (Ref).

Refer to adult dosing; use reduced initial dose.

Dosing: Pediatric

(For additional information see "Droperidol: Pediatric drug information")

Dosage must be individualized based on age, body weight, underlying medical conditions, physical status, concomitant medications, type of anesthesia, and surgical procedure.

Agitation, acute

Agitation, acute: Limited data available; dosing regimen variable; optimal dose not established (Ref):

Weight-directed dosing: Children ≥ 7 years and Adolescents: IM, IV: 0.1 to 0.2 mg/kg/dose; maximum dose: 10 mg/dose; dosing based on management of prehospital agitation by emergency medical services (n=96) (Ref). **Note:** Although not described in the trial, may also administer IV if route is feasible (Ref).

Fixed dosing:

Children ≥ 7 years and Adolescents: May repeat dose in 15 to 30 minutes if no response (Ref).

<34 kg: IM, IV: 0.625 mg.

34 to 57 kg: IM, IV: 1.25 mg.

>57 to 68 kg: IM, IV: 1.875 mg.

>68 kg: IM, IV: 2.5 mg.

Dosing reported in an inpatient psychiatric setting to manage agitation in patients (n=26) with an underlying psychiatric disorder. **Note:** Although not described in the trial, may also administer IV if route is feasible (Ref).

Postoperative nausea and vomiting; rescue treatment

Postoperative nausea and vomiting (PONV); rescue treatment: Note: Not routinely used for prevention; however, droperidol may be necessary as rescue therapy (Ref).

Children ≥ 2 years and Adolescents: IM, IV: 0.01 to 0.075 mg/kg/dose; maximum dose: 1.25 mg/dose; administer additional doses with extreme caution and only if potential benefit outweighs risks (Ref).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Pediatric

There are no dosage adjustments provided in the manufacturer's labeling; use with caution.

Dosing: Liver Impairment: Pediatric

There are no dosage adjustments provided in the manufacturer's labeling; use with caution.

Adverse Reactions

The following adverse drug reactions are derived from product labeling unless otherwise specified.

Postmarketing:

Cardiovascular: Cardiac arrhythmia (including ventricular arrhythmia), hypotension (Chase 2002), prolonged QT interval on ECG (Cole 2020), tachycardia, torsades de pointes (Cole 2020), ventricular tachycardia (Takechi 2021)

Hypersensitivity: Anaphylaxis

Nervous system: Anxiety (Lim 1999), chills, depression, dizziness (Henzi 2000), drowsiness (Henzi 2000), dysphoria (Lim 1999), extrapyramidal reaction (including akathisia, dystonia) (Melnick 1988), hallucination, hyperactive behavior, neuroleptic malignant syndrome (So 2001), restlessness (Henzi 2000), sedated state (Henzi 2000), shivering, vertigo (Henzi 2000)

Neuromuscular & skeletal: Laryngospasm

Respiratory: Bronchospasm, respiratory depression (Ramsden 2022)

Contraindications

Hypersensitivity to droperidol or any component of the formulation; known or suspected QT prolongation, including congenital long QT syndrome (prolonged QT_c is defined as >440 msec in males or >450 msec in females)

Canadian labeling: Additional contraindications (not in US labeling): Not for use in children ≤ 2 years of age

Warnings/Precautions

Concerns related to adverse effects:

- Arrhythmias: Use extreme caution in patients with bradycardia (<50 bpm), cardiac disease, concurrent MAO inhibitor therapy, Class I and Class III antiarrhythmics or other drugs known to prolong QT interval, and electrolyte disturbances (hypokalemia or hypomagnesemia), including concomitant drugs which may alter electrolytes (diuretics).

- Anticholinergic effects: May cause anticholinergic effects (constipation, xerostomia, blurred vision, urinary retention); use with caution in patients with decreased gastrointestinal motility, urinary retention, BPH, xerostomia, or visual problems. Relative to other antipsychotics, aripiprazole has a low potency of cholinergic blockade (Richelson 1999).
- CNS depression: May cause CNS depression, which may impair physical or mental abilities; patients must be cautioned about performing tasks that require mental alertness (eg, operating machinery or driving).
- Esophageal dysmotility/aspiration: Antipsychotic use has been associated with esophageal dysmotility and aspiration; risk increases with age. Use with caution in patients at risk for aspiration pneumonia (eg, Alzheimer disease), particularly in patients >75 years (Herzig 2017; Maddalena 2004).
- Extrapyramidal symptoms: May cause extrapyramidal symptoms (EPS), including pseudoparkinsonism, acute dystonic reactions, akathisia, and tardive dyskinesia (risk of these reactions is generally much lower relative to typical/conventional antipsychotics; frequencies reported are similar to placebo). Risk of dystonia (and possibly other EPS) may be greater with increased doses, use of conventional antipsychotics, males, and younger patients. Factors associated with greater vulnerability to tardive dyskinesia include older in age, female gender combined with postmenopausal status, Parkinson disease, pseudoparkinsonism symptoms, affective disorders (particularly major depressive disorder), concurrent medical diseases such as diabetes, previous brain damage, alcohol use disorder, poor treatment response, and use of high doses of antipsychotics (APA [Keepers 2020]; Soares-Weiser 2007). Consider therapy discontinuation with signs/symptoms of tardive dyskinesia.
- Falls: May increase the risk for falls due to somnolence, orthostatic hypotension, and motor or sensory instability (Landi 2005; Seppala 2018).
- Hyperprolactinemia: May increase prolactin levels; clinical significance of hyperprolactinemia in patients with breast cancer or other prolactin-dependent tumors is unknown (Bargiotta 2013; Froes Brandao 2016).
- Neuroleptic malignant syndrome (NMS): Use may be associated with neuroleptic malignant syndrome (NMS); monitor for mental status changes, fever, muscle rigidity and/or autonomic instability.
- Orthostatic hypotension: May cause orthostatic hypotension; use with caution in patients at risk of this effect or in those who would not tolerate transient hypotensive episodes (cerebrovascular disease, cardiovascular disease, hypovolemia, or concurrent medication use which may predispose to hypotension/bradycardia).
- Temperature regulation: Impaired core body temperature regulation may occur; caution with strenuous exercise, heat exposure, dehydration, and concomitant medication possessing anticholinergic effects.

Disease-related concerns:

- Hepatic impairment: Use with caution in patients with severe hepatic impairment.
- Pheochromocytoma: Use with caution in patients with pheochromocytoma; severe hypertension and/or tachycardia may occur.
- Renal impairment: Use with caution in patients with renal impairment.
- Seizures: Use with caution in patients at risk of seizures, including those with a history of seizures, head trauma, brain damage, alcohol use disorder, or concurrent therapy with medications which may lower seizure threshold (Chase 2002).

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution, Injection [preservative free]:

Generic: 2.5 mg/mL (1 mL, 2 mL)

Generic Equivalent Available: US

Yes

Pricing: US

Solution (droPERidol Injection)

2.5 mg/mL (per mL): \$5.81 - \$6.38

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Administration: Adult

IM, IV: Administer IM or IV; according to the manufacturer, IV push administration should be slow. Generally administered undiluted (Ref).

Preparation for Administration: Adult

If necessary, may dilute in NS; concentrations of 0.02 to 0.875 mg/mL have been reported (Ref).

If necessary, may dilute in D5W; concentrations of 0.025 mg/mL have been reported (Ref).

Refer to institutional protocol.

Administration: Pediatric

Parenteral: Administer undiluted IM or by slow IV injection over 2 to 5 minutes.

Storage/Stability

Store at 20°C to 25°C (68°F to 77°F); excursions permitted to 15°C to 30°C (59°F to 86°F). Protect from light. Solutions diluted in NS or D5W are stable at room temperature for up to 7 days in PVC bags or glass bottles. Solutions diluted in LR are stable at room temperature for 24 hours in PVC bags and up to 7 days in glass bottles.

Use: Labeled Indications

Postoperative nausea/vomiting (PONV): Prevention and/or treatment of nausea and vomiting from surgical and diagnostic procedures

Use: Off-Label: Adult

Acute undifferentiated agitation; Cannabinoid hyperemesis syndrome

Medication Safety Issues

Sound-alike/look-alike issues:

DroPERidol may be confused with droNABinol

Older Adult: High-Risk Medication:

Beers Criteria: Antipsychotics are identified in the Beers Criteria as potentially inappropriate medications to be avoided in patients 65 years and older due to an increased risk of stroke and a greater rate of cognitive decline and mortality in patients with dementia. Evidence also suggests there may be an increased risk of mortality with use independent of dementia. Avoid antipsychotics for behavioral problems associated with dementia or delirium unless alternative nonpharmacologic therapies have failed and patient may harm self or others. In addition, antipsychotics should be used with caution

in older adults due to their potential to cause or exacerbate syndrome of inappropriate antidiuretic hormone secretion (SIADH) or hyponatremia; monitor sodium closely with initiation or dosage adjustments in older adults. Use of antipsychotics may be appropriate for labeled indications including schizophrenia, bipolar disorder, Parkinson disease psychosis, adjunctive therapy in major depressive disorder, or for short-term use as an antiemetic (Beers Criteria [AGS 2023]).

Pediatric patients: High-risk medication:

KIDs List: Dopamine antagonists, including droperidol, are identified on the Key Potentially Inappropriate Drugs in Pediatrics (KIDs) list. Avoid use in patients ≤ 18 years of age due to risk of acute dystonic reactions (eg, oculogyric crisis, torticollis) (strong recommendation; high quality level of evidence) and risk of hyperprolactinemia (weak recommendation; high quality of evidence) (PPA [McPherson 2025]).

Metabolism/Transport Effects

None known.

Drug Interactions

(For additional information: Launch drug interactions program)

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within “CYP3A4 Inducers [Strong]” are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the “Launch drug interactions program” link above.

Agents With Seizure Threshold Lowering Potential: May increase adverse/toxic effects of DroPERidol. Specifically, the risk of seizures may be increased. *Risk C: Monitor*

Alcohol (Ethyl): DroPERidol may increase CNS depressant effects of Alcohol (Ethyl). Management: Limit alcohol consumption during droperidol use. *Risk D: Consider Therapy Modification*

Alizapride: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Allopurinol: CNS Depressants may increase sedative effects of Allopurinol. *Risk C: Monitor*

Amifampridine: Agents With Seizure Threshold Lowering Potential may increase neuroexcitatory and/or seizure-potentiating effects of Amifampridine. *Risk C: Monitor*

Amisulpride (Injection): May increase QTc-prolonging effects of DroPERidol. *Risk X: Avoid*

Amisulpride (Oral): Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Amisulpride (Oral). Specifically, the risk of seizures may be increased. *Risk C: Monitor*

Amisulpride (Oral): May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Amisulpride (Oral): May increase QTc-prolonging effects of QT-prolonging Agents (Moderate Risk). *Risk C: Monitor*

Anti-Parkinson Agents (Dopamine Agonist): Antipsychotic Agents (First Generation [Typical]) may decrease therapeutic effects of Anti-Parkinson Agents (Dopamine Agonist). Management: Avoid concomitant therapy if possible. If antipsychotic use is necessary, consider using atypical antipsychotics such as clozapine, quetiapine, or ziprasidone at lower initial doses, or a non-dopamine antagonist (eg, pimavanserin). *Risk D: Consider Therapy Modification*

ARIPrazole Lauroxil: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of ARIPrazole Lauroxil. Specifically, the risk of seizures may be increased. *Risk C: Monitor*

ARIPrazole: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of ARIPrazole. Specifically, the risk of seizures may be increased. *Risk C: Monitor*

Azelastine (Nasal): May increase CNS depressant effects of CNS Depressants. *Risk X: Avoid*

Azithromycin (Systemic): May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

Barbiturates: CNS Depressants may increase CNS depressant effects of Barbiturates. *Risk C: Monitor*

Benperidol: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Benperidol. Specifically, the risk of seizures may be increased. *Risk C: Monitor*

Benperidol: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Benzgalantamine-Galantamine: May increase neurotoxic (central) effects of Antipsychotic Agents. *Risk C: Monitor*

Blonanserin: CNS Depressants may increase CNS depressant effects of Blonanserin. Management: Use caution if coadministering blonanserin and CNS depressants; dose reduction of the other CNS depressant may be required. Strong CNS depressants should not be coadministered with blonanserin. *Risk D: Consider Therapy Modification*

Bornaprine: May increase adverse/toxic effects of Antipsychotic Agents. Specifically, tardive dyskinesia symptoms may be potentiated. *Risk C: Monitor*

Brexipiprazole: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Brexpiprazole. Specifically, the risk of seizures may be increased. *Risk C: Monitor*

Brimonidine (Topical): May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Bromopride: DroPERidol may increase adverse/toxic effects of Bromopride. *Risk X: Avoid*

Bromperidol: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Bromperidol. Specifically, the risk of seizures may be increased. *Risk C: Monitor*

Bromperidol: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Buprenorphine: CNS Depressants may increase CNS depressant effects of Buprenorphine. Management: Avoid coadministration of buprenorphine and other CNS depressants if possible. If combined, consider adjustments to induction, increased monitoring, and co-prescription of an opioid overdose reversal agent. *Risk D: Consider Therapy Modification*

BuPROPion: May increase neuroexcitatory and/or seizure-potentiating effects of Agents With Seizure Threshold Lowering Potential. *Risk C: Monitor*

BusPIRone: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Cabergoline: May decrease therapeutic effects of Antipsychotic Agents. *Risk X: Avoid*

Carbinoxamine: May increase CNS depressant effects of CNS Depressants. *Risk X: Avoid*

Cariprazine: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Cariprazine. Specifically, the risk of seizures may be increased. *Risk C: Monitor*

Ceritinib: May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

Cetirizine (Systemic): May increase CNS depressant effects of CNS Depressants. Management: Consider avoiding this combination if possible. If required, monitor for excessive sedation or CNS depression, limit the dose and duration of combination therapy, and consider CNS depressant dose reductions. *Risk D: Consider Therapy Modification*

Chloral Hydrate-Chloral Betaine: CNS Depressants may increase CNS depressant effects of Chloral Hydrate-Chloral Betaine. Management: Consider alternatives to the use of chloral hydrate or chloral betaine and additional CNS depressants. If combined, consider a dose reduction of either agent and monitor closely for enhanced CNS depressive effects. *Risk D: Consider Therapy Modification*

Chlormethiazole: May increase CNS depressant effects of CNS Depressants. Management: Monitor closely for evidence of excessive CNS depression. The chlormethiazole labeling states that an appropriately reduced dose should be used if such a combination must be used. *Risk D: Consider Therapy Modification*

Chlorphenesin Carbamate: May increase adverse/toxic effects of CNS Depressants. *Risk C: Monitor*

ChlorproMAZINE: DroPERidol may increase QTc-prolonging effects of ChlorproMAZINE. DroPERidol may increase CNS depressant effects of ChlorproMAZINE. Management: Consider alternatives to this drug combination. If combined, dose reductions are recommended. Monitor for additive toxicities such as QTc interval prolongation, ventricular arrhythmias, and CNS depression. *Risk D: Consider Therapy Modification*

Clarithromycin: QT-prolonging Antipsychotics (Moderate Risk) may increase QTc-prolonging effects of Clarithromycin. Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

CloZAPine: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of CloZAPine. Specifically, the risk of seizures may be increased. *Risk C: Monitor*

CloZAPine: CNS Depressants may increase CNS depressant effects of CloZAPine. *Risk C: Monitor*

CloZAPine: QT-prolonging Antipsychotics (Moderate Risk) may increase QTc-prolonging effects of CloZAPine. Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

CNS Depressants: DroPERidol may increase CNS depressant effects of CNS Depressants. Management: Consider dose reductions of droperidol or of other CNS agents (eg, opioids, barbiturates) with concomitant use. *Risk D: Consider Therapy Modification*

Daridorexant: May increase CNS depressant effects of CNS Depressants. Management: Dose reduction of daridorexant and/or any other CNS depressant may be necessary. Use of daridorexant with alcohol is not recommended, and the use of daridorexant with any other drug to treat insomnia is not recommended. *Risk D: Consider Therapy Modification*

Dexmedetomidine: CNS Depressants may increase CNS depressant effects of Dexmedetomidine. Management: Monitor for increased CNS depression during coadministration of dexmedetomidine and CNS depressants, and consider dose reductions of either agent to avoid excessive CNS depression. *Risk D: Consider Therapy Modification*

Dexmethylphenidate-Methylphenidate: Antipsychotic Agents may increase adverse/toxic effects of Dexmethylphenidate-Methylphenidate. Dexmethylphenidate-Methylphenidate may increase adverse/toxic effects of Antipsychotic Agents. Specifically, the risk of extrapyramidal symptoms may be increased when these agents are combined. *Risk C: Monitor*

Difelikefalin: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Dihydralazine: CNS Depressants may increase hypotensive effects of Dihydralazine. *Risk C: Monitor*

Dimethindene (Topical): May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Diphenoxylate: May increase CNS depressant effects of CNS Depressants. Management: Avoid coadministration of diphenoxylate/atropine with other CNS depressants if possible. If coadministration cannot be avoided, monitor patients closely for CNS depression (eg, sedation, somnolence) with any combined use. *Risk D: Consider Therapy Modification*

Domperidone: QT-prolonging Agents (Moderate Risk) may increase QTc-prolonging effects of Domperidone. *Risk X: Avoid*

Donepezil: May increase neurotoxic (central) effects of Antipsychotic Agents. *Risk C: Monitor*

Emedastine (Systemic): May increase CNS depressant effects of CNS Depressants. Management: Consider avoiding this combination if possible. If required, monitor for excessive sedation or CNS

depression, limit the dose and duration of combination therapy, and consider CNS depressant dose reductions. *Risk C: Monitor*

Encorafenib: May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

Entacapone: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Eszopiclone: CNS depressant effects of CNS Depressants and Eszopiclone may increase with coadministration. CNS Depressants may increase adverse/toxic effects of Eszopiclone. Specifically, the risk of daytime psychomotor impairment may be increased. Management: Consider reducing the dose of eszopiclone or other CNS depressants if these agents are used concomitantly, and monitor for excessive daytime psychomotor impairment with any combined use. Use of other sedative-hypnotics during the night is not recommended. *Risk D: Consider Therapy Modification*

Fexinidazole: QT-prolonging Antipsychotics (Moderate Risk) may increase QTc-prolonging effects of Fexinidazole. Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

Flunarizine: CNS Depressants may increase CNS depressant effects of Flunarizine. *Risk X: Avoid*

Fluorouracil Products: QT-prolonging Antipsychotics (Moderate Risk) may increase QTc-prolonging effects of Fluorouracil Products. Management: Monitor for QTc interval prolongation and ventricular arrhythmias, including torsades de pointes when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

Guanethidine: Antipsychotic Agents may decrease therapeutic effects of Guanethidine. *Risk C: Monitor*

Huperzine A: May increase neurotoxic (central) effects of Antipsychotic Agents. *Risk C: Monitor*

Hydroxyzine: May increase CNS depressant effects of CNS Depressants. Management: Reduce the dose of hydroxyzine or other CNS depressants if these agents are combined due to the potential for increased CNS depressant effects. *Risk D: Consider Therapy Modification*

Iohexol: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Iohexol. Specifically, the risk for seizures may be increased. Management: Discontinue agents that may lower the seizure threshold 48 hours prior to intrathecal use of Iohexol. Wait at least 24 hours after the procedure to resume such agents. In nonelective procedures, consider use of prophylactic antiseizure drugs. *Risk D: Consider Therapy Modification*

Iomeprol: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Iomeprol. Specifically, the risk for seizures may be increased. Management: Discontinue agents that may lower the seizure threshold 48 hours prior to intrathecal use of Iomeprol. Wait at least 24 hours after the procedure to resume such agents. In nonelective procedures, consider use of prophylactic antiseizure drugs. *Risk D: Consider Therapy Modification*

Iopamidol: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Iopamidol. Specifically, the risk for seizures may be increased. Management: Discontinue agents that may lower the seizure threshold 48 hours prior to intrathecal use of Iopamidol. Wait at least 24 hours after the procedure to resume such agents. In nonelective procedures, consider use of prophylactic antiseizure drugs. *Risk D: Consider Therapy Modification*

Isocarboxazid: CNS Depressants may increase adverse/toxic effects of Isocarboxazid. *Risk X: Avoid*

Ixabepilone: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Kava Kava: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Ketotifen (Systemic): May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Kratom: May increase CNS depressant effects of CNS Depressants. *Risk X: Avoid*

Lemborexant: May increase CNS depressant effects of CNS Depressants. Management: Dosage adjustments of lemborexant and of concomitant CNS depressants may be necessary when administered together because of potentially additive CNS depressant effects. Close monitoring for CNS depressant effects is necessary. *Risk D: Consider Therapy Modification*

Levocetirizine: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Levoketoconazole: QT-prolonging Agents (Moderate Risk) may increase QTc-prolonging effects of Levoketoconazole. *Risk X: Avoid*

Lindane: Agents With Seizure Threshold Lowering Potential may increase neuroexcitatory and/or seizure-potentiating effects of Lindane. *Risk C: Monitor*

Lithium: May increase neurotoxic effects of Antipsychotic Agents. Lithium may decrease serum concentrations of Antipsychotic Agents. Specifically noted with chlorpromazine. *Risk C: Monitor*

Lofexidine: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Lonafarnib: May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

Loxapine: CNS Depressants may increase CNS depressant effects of Loxapine. Management: Consider reducing the dose of CNS depressants administered concomitantly with loxapine due to an increased risk of respiratory depression, sedation, hypotension, and syncope. *Risk D: Consider Therapy Modification*

Magnesium Sulfate: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Mequitazine: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Metergoline: Antipsychotic Agents may decrease therapeutic effects of Metergoline. Metergoline may decrease therapeutic effects of Antipsychotic Agents. *Risk C: Monitor*

Metergoline: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Methadone: DroPERidol may increase QTc-prolonging effects of Methadone. DroPERiaol may increase CNS depressant effects of Methadone. Management: Consider alternatives. If combined, dose reductions are recommended. Monitor for additive toxicities such as QTc interval prolongation, ventricular arrhythmias, and CNS depression. Patients with additional risk factors are at even higher risk. *Risk D: Consider Therapy Modification*

Methotrimeprazine: CNS Depressants may increase CNS depressant effects of Methotrimeprazine. Methotrimeprazine may increase CNS depressant effects of CNS Depressants. Management: Reduce the usual dose of CNS depressants by 50% if starting methotrimeprazine until the dose of methotrimeprazine is stable. Monitor patient closely for evidence of CNS depression. *Risk D: Consider Therapy Modification*

Metoclopramide: DroPERiaol may increase adverse/toxic effects of Metoclopramide. *Risk X: Avoid*

MetyroSINE: CNS Depressants may increase sedative effects of MetyroSINE. *Risk C: Monitor*

MetyroSINE: May increase adverse/toxic effects of Antipsychotic Agents. Specifically, the risk for extrapyramidal symptoms and excessive sedation may be increased. *Risk C: Monitor*

Minocycline (Systemic): May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Moxonidine: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Nabilone: May increase CNS depressant effects of CNS Depressants. *Risk X: Avoid*

Noscapine: CNS Depressants may increase adverse/toxic effects of Noscapine. *Risk X: Avoid*

Olopatadine (Nasal): May increase CNS depressant effects of CNS Depressants. *Risk X: Avoid*

Ondansetron: May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation, ventricular arrhythmias, including torsades de pointes, when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

Opicapone: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Opioid Agonists: CNS Depressants may increase CNS depressant effects of Opioid Agonists. Management: Avoid concomitant use of opioid agonists and benzodiazepines or other CNS depressants when possible. These agents should only be combined if alternative treatment options are inadequate. If combined, limit the dosage and duration of each drug. *Risk D: Consider Therapy Modification*

Opipramol: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Oxomemazine: May increase CNS depressant effects of CNS Depressants. *Risk X: Avoid*

Oxybate (Mixed Salts and Sodium Salt): CNS Depressants may increase CNS depressant effects of Oxybate (Mixed Salts and Sodium Salt). Management: Consider alternatives to this combination when possible. If combined, dose reduction or discontinuation of one or more CNS depressants (including the oxybate salt product) should be considered. Interrupt oxybate salt treatment during short-term opioid use *Risk D: Consider Therapy Modification*

Paraldehyde: CNS Depressants may increase CNS depressant effects of Paraldehyde. *Risk X: Avoid*

Pentamidine (Systemic): May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

Periciazine: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Periciazine. Specifically, the risk of seizures may be increased. *Risk C: Monitor*

Periciazine: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Pimozide: May increase QTc-prolonging effects of QT-prolonging Agents (Moderate Risk). *Risk X: Avoid*

Pipamperone: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Pipamperone. Specifically, the risk of seizures may be increased. *Risk X: Avoid*

Piperaquine: QT-prolonging Agents (Moderate Risk) may increase QTc-prolonging effects of Piper-
aquine. *Risk X: Avoid*

Piribedil: Antipsychotic Agents may decrease therapeutic effects of Piribedil. Piribedil may decrease therapeutic effects of Antipsychotic Agents. Management: Concurrent use of piribedil with antipsychotic neuroleptics, other than clozapine, is not recommended. Antiemetic antipsychotics are contraindicated in combination with piribedil. *Risk D: Consider Therapy Modification*

Polyethylene Glycol-Electrolyte Solution: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Polyethylene Glycol-Electrolyte Solution. Specifically, the risk of seizure may be increased. *Risk C: Monitor*

Procarbazine: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Promazine: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Promazine. Specifically, the risk of seizures may be increased. *Risk C: Monitor*

Promazine: CNS Depressants may increase CNS depressant effects of Promazine. *Risk C: Monitor*

Promethazine: CNS Depressants may increase CNS depressant effects of Promethazine. Management: Discontinue or reduce the dose of other CNS depressants during promethazine therapy. *Risk D: Consider Therapy Modification*

Propofol: DroPERidol may increase CNS depressant effects of Propofol. Propofol may increase QTc-prolonging effects of DroPERidol. Management: Consider dose reductions of droperidol or propofol

during coadministration due to additive effects on CNS depression. Also monitor for QTc prolongation and ventricular arrhythmia. *Risk D: Consider Therapy Modification*

QT-prolonging Agents (Highest Risk): May increase QTc-prolonging effects of DroPERidol. Management: Consider alternatives to this combination. Patients with other risk factors (eg, older age, female sex, bradycardia, hypokalemia, hypomagnesemia, heart disease, and higher drug concentrations) are likely at greater risk for these toxicities. *Risk D: Consider Therapy Modification*

QT-prolonging Antidepressants (Moderate Risk): QT-prolonging Antipsychotics (Moderate Risk) may increase QTc-prolonging effects of QT-prolonging Antidepressants (Moderate Risk). QT-prolonging Antipsychotics (Moderate Risk) may increase serotonergic effects of QT-prolonging Antidepressants (Moderate Risk). This could result in serotonin syndrome. Management: Monitor for QTc interval prolongation, ventricular arrhythmias, and serotonin syndrome/serotonin toxicity (SS/ST) or NMS when these agents are combined. Patients with additional risk factors for QTc prolongation or SS/ST may be at even higher risk. *Risk C: Monitor*

QT-prolonging Antipsychotics (Moderate Risk): May increase QTc-prolonging effects of DroPERidol. QT-prolonging Antipsychotics (Moderate Risk) may increase CNS depressant effects of DroPERidol. Management: Consider dose reductions and monitor for CNS depression, QTc interval prolongation, and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk D: Consider Therapy Modification*

QT-prolonging Class IC Antiarrhythmics (Moderate Risk): May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

QT-Prolonging Inhalational Anesthetics (Moderate Risk): DroPERidol may increase CNS depressant effects of QT-Prolonging Inhalational Anesthetics (Moderate Risk). DroPERidol may increase QTc-prolonging effects of QT-Prolonging Inhalational Anesthetics (Moderate Risk). Management: Consider dose reductions and monitor for QTc interval prolongation and ventricular arrhythmias, including torsades de pointes when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk D: Consider Therapy Modification*

QT-prolonging Kinase Inhibitors (Moderate Risk): QT-prolonging Antipsychotics (Moderate Risk) may increase QTc-prolonging effects of QT-prolonging Kinase Inhibitors (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

QT-prolonging Miscellaneous Agents (Moderate Risk): May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

QT-prolonging Moderate CYP3A4 Inhibitors (Moderate Risk): May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

QT-prolonging Quinolone Antibiotics (Moderate Risk): May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

Quinagolide: Antipsychotic Agents may decrease therapeutic effects of Quinagolide. *Risk C: Monitor*

Rilmenidine: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Rivastigmine: May increase neurotoxic (central) effects of Antipsychotic Agents. *Risk C: Monitor*

Ropeginterferon Alfa-2b: CNS Depressants may increase adverse/toxic effects of Ropeginterferon Alfa-2b. Specifically, the risk of neuropsychiatric adverse effects may be increased. Management: Avoid coadministration of ropeginterferon alfa-2b and other CNS depressants. If this combination

cannot be avoided, monitor patients for neuropsychiatric adverse effects (eg, depression, suicidal ideation, aggression, mania). *Risk D: Consider Therapy Modification*

Scopolamine: CNS Depressants may increase CNS depressant effects of Scopolamine. Management: Avoid coadministration of scopolamine with other CNS depressants if possible. If coadministration is required, monitor patients for excessive CNS depression. *Risk D: Consider Therapy Modification*

Serotonergic Agents (High Risk): May increase adverse/toxic effects of Antipsychotic Agents. Specifically, serotonergic agents may enhance dopamine blockade, possibly increasing the risk for neuroleptic malignant syndrome. Antipsychotic Agents may increase serotonergic effects of Serotonergic Agents (High Risk). This could result in serotonin syndrome. *Risk C: Monitor*

Sertindole: May increase QTc-prolonging effects of QT-prolonging Agents (Moderate Risk). *Risk X: Avoid*

Sodium Phosphates: Agents With Seizure Threshold Lowering Potential may increase adverse/toxic effects of Sodium Phosphates. Specifically, the risk of seizure or loss of consciousness may be increased in patients with significant sodium phosphate-induced fluid or electrolyte abnormalities. *Risk C: Monitor*

Sulpiride: Antipsychotic Agents may increase adverse/toxic effects of Sulpiride. *Risk X: Avoid*

Thalidomide: CNS Depressants may increase CNS depressant effects of Thalidomide. *Risk X: Avoid*

Thiopental: DroPERidol may increase therapeutic effects of Thiopental. Management: Consider thiopental dose reduction when used concomitantly with droperidol. Monitor patient response to treatment closely if using this combination. *Risk D: Consider Therapy Modification*

Thioridazine: QT-prolonging Agents (Moderate Risk) may increase QTc-prolonging effects of Thioridazine. *Risk X: Avoid*

Tradipitant: CNS Depressants may increase CNS depressant effects of Tradipitant. *Risk C: Monitor*

Trimethobenzamide: CNS Depressants may increase CNS depressant effects of Trimethobenzamide. Management: Avoid coadministration of trimethobenzamide with other CNS depressants if possible. If coadministration cannot be avoided, monitor patients for CNS adverse effects (eg, depression, disorientation, seizures). *Risk D: Consider Therapy Modification*

Valerian: May increase CNS depressant effects of CNS Depressants. *Risk C: Monitor*

Voriconazole: May increase QTc-prolonging effects of QT-prolonging Antipsychotics (Moderate Risk). Management: Monitor for QTc interval prolongation and ventricular arrhythmias when these agents are combined. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk C: Monitor*

Zaleplon: CNS Depressants may increase CNS depressant effects of Zaleplon. Management: Coadministration of zaleplon with other sedative-hypnotics at bedtime or in the middle of the night is not recommended. If zaleplon is coadministered with other CNS depressants at other times, monitor for CNS depression; dose reductions may be required. *Risk D: Consider Therapy Modification*

Ziprasidone: DroPERidol may increase QTc-prolonging effects of Ziprasidone. Management: Consider alternatives to this drug combination. If combined, monitor for QTc interval prolongation and ventricular arrhythmias. Patients with additional risk factors for QTc prolongation may be at even higher risk. *Risk X: Avoid*

Zolpidem: CNS Depressants may increase CNS depressant effects of Zolpidem. Management: Reduce the Intermezzo brand sublingual zolpidem adult dose to 1.75 mg for men who are also receiving other CNS depressants. No such dose change is recommended for women. Avoid use with other CNS depressants at bedtime; avoid use with alcohol. *Risk D: Consider Therapy Modification*

Zuclopenthixol: May increase CNS depressant effects of CNS Depressants. Management: Avoid use of high-dose hypnotics or other CNS depressants together with zuclopenthixol. *Risk D: Consider Therapy Modification*

Zuranolone: May increase CNS depressant effects of CNS Depressants. Management: Consider alternatives to the use of zuranolone with other CNS depressants or alcohol. If combined, consider a zuranolone dose reduction and monitor patients closely for increased CNS depressant effects. *Risk D: Consider Therapy Modification*

Pregnancy Considerations

Droperidol has been evaluated for the adjunctive management of hyperemesis gravidarum (Ferreira 2003; Nageotte 1996); however, use for the treatment of persistent symptoms of nausea and vomiting in pregnancy is not recommended (ACOG 189 2018).

Breastfeeding Considerations

It is not known if droperidol is present in breast milk.

Adverse events were observed in breastfed infants following maternal administration during cesarean delivery (based on Neurologic and Adaptive Capacity Scores); however, additional study is needed (Bonhomme 2002). Theoretically, droperidol may interfere with milk production. Although data is limited, infant exposure following a single maternal dose will limit the potential for adverse events in a breastfed infant (Spigset 1994). The manufacturer recommends that caution be exercised when administering droperidol to breastfeeding women.

Monitoring Parameters

Frequency of Antipsychotic Monitoring in Droperidol^{a,b}

Monitoring Parameter

Frequency of Monitoring

Comments

^a For all monitoring parameters, it is appropriate to check at baseline and when clinically relevant (based on symptoms or suspected adverse reactions) in addition to the timeline.

^b ADA 2004; APA [Keepers 2020]; Landi 2005; Seppala 2018; manufacturer's labeling.

^c Risk factors for extrapyramidal symptoms (EPS) include prior history of EPS, high doses of antipsychotics, young age (children and adolescents at higher risk than adults), and dopaminergic affinity of individual antipsychotic.

^d Risk factors for tardive dyskinesia include age >55 years; females; White or African ethnicity; presence of a mood disorder, intellectual disability, or CNS injury; and past or current EPS.

^e Cardiac risk factors include congenital long QT syndrome, structural or functional cardiac disease, bradycardia, family history of sudden cardiac death

Adherence

Every visit

Blood chemistries (electrolytes, renal function, liver function, TSH)

As clinically indicated

CBC

As clinically indicated

Check frequently during the first few months of therapy in patients with preexisting low WBC or history of drug-induced leukopenia/neutropenia

ECG

Continue monitoring for 2 to 3 hours following administration, and as clinically indicated

Check after significant dose increase or new QTc-prolonging medication if there are cardiac risk factors.^e

Extrapyramidal symptoms

Every visit; 4 weeks after initiation and dose change; annually. Use a formalized rating scale at least annually or every 6 months if high risk.^c

Fall risk

As clinically indicated

Evaluate regularly in patients ≥ 60 years of age.

Fasting plasma glucose/HbA_{1c}

4 months after initiation; annually

Check more frequently than annually if abnormal. Follow diabetes guidelines.

Lipid panel

4 months after initiation; annually

Check more frequently than annually if abnormal. Follow lipid guidelines.

Mental status and alertness

Every visit

Metabolic syndrome history

Annually

Evaluate for personal and family history of obesity, diabetes, dyslipidemia, hypertension, or cardiovascular disease.

Ocular exam

As clinically indicated

Particularly important for those taking thioridazine or chlorpromazine, or those with diabetes and other conditions that impact sight.

Prolactin

Ask about symptoms at every visit until dose is stable. Check prolactin level if symptoms are reported.

Hyperprolactinemia symptoms: Changes in menstruation, libido, gynecomastia, development of galactorrhea, and erectile and ejaculatory function.

Tardive dyskinesia

Every visit; annually. Use a formalized rating scale at least annually or every 6 months if high risk.^d

Vital signs (BP, orthostatics, temperature, pulse, signs of infection)

As clinically indicated

Weight/Height/BMI

Every visit for first 6 months, then quarterly

Consider monitoring waist circumference at baseline and annually, especially in patients with or at risk for metabolic syndrome.

Consider changing antipsychotic if BMI increases by ≥ 1 unit.

Mechanism of Action

Droperidol is a butyrophenone antipsychotic; antiemetic effect is a result of blockade of dopamine stimulation of the chemoreceptor trigger zone. Other effects include alpha-adrenergic blockade, peripheral vascular dilation, and reduction of the pressor effect of epinephrine resulting in hypotension and decreased peripheral vascular resistance; may also reduce pulmonary artery pressure

Pharmacokinetics (Adult Data Unless Noted)

Onset of action: 3 to 10 minutes

Peak effect: ~30 minutes

Duration: 2 to 4 hours, may extend to 12 hours

Absorption: IM: Rapid (Cressman 1973)

Distribution: Crosses blood-brain barrier (McKeage 2006)

V_d : Children: ~0.6 L/kg; Adults: ~1.5 L/kg (McKeage 2006)

Protein binding: 85% to 90% (McKeage 2006)

Metabolism: Hepatic, to *p*-fluorophenylacetic acid, benzimidazolone, *p*-hydroxypiperidine

Half-life elimination: Children ~ 1.7 hours; Adults: ~2 hours (McKeage 2006)

Excretion: Urine (75%, <1% as unchanged drug); feces (22%, 11% as unchanged drug) (McKeage 2006)

Patient Counseling Points

(For additional information see "Droperidol: Patient drug information")

What is this drug used for?

- It is used to prevent upset stomach and throwing up.

All drugs may cause side effects. However, many people have no side effects or only have minor side effects. Call your doctor or get medical help if any of these side effects or any other side effects bother you or do not go away:

- Feeling nervous and excitable
- Feeling sleepy
- Restlessness

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

- Trouble controlling body movements, twitching, change in balance, trouble swallowing or speaking
- Fast or abnormal heartbeat
- Very bad dizziness or passing out
- Hallucinations (seeing or hearing things that are not there)
- Chest pain or pressure
- Change in how you act
- Neuroleptic malignant syndrome like fever, muscle cramps or stiffness, dizziness, very bad headache, confusion, change in thinking, fast heartbeat, heartbeat that does not feel normal, or sweating a lot
- Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

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does not endorse any medicine as safe, effective, or approved for treating any patient or health condition. This is only a limited summary of general information about the medicine's uses from the patient education leaflet and is not intended to be comprehensive. This limited summary does NOT include all information available about the possible uses, directions, warnings, precautions, interactions, adverse effects, or risks that may apply to this medicine. This information is not intended to provide medical advice, diagnosis or treatment and does not replace information you receive from the healthcare provider. For a more detailed summary of information about the risks and benefits of using this medicine, please speak with your healthcare provider and review the entire patient education leaflet.

Brand Names: International

International Brand Names by Country

For country code abbreviations (show table)

- (AT) Austria: Droperiaol kalceks | Ponveridol | Xomolix;
- (AU) Australia: DROPERIDOL SXP;
- (BE) Belgium: Dehydrobenzperidol | Droperiaol kalceks;
- (CH) Switzerland: Droperiaol sintetica;
- (CZ) Czech Republic: Xomolix;
- (DE) Germany: Droperidol carino | Droperiaol carinopharm | Droperiaol panpharma | Droperidol rotexmedica | Ponveridol | Xomolix;
- (EE) Estonia: Droperiaol aguettant | Droperidol panpharma;
- (ES) Spain: Droperiaol Hikma | Droperiaol kalceks | Xomolix;
- (FI) Finland: Dehydrobenzperidol | Droperiaol sintetica;
- (FR) France: Droleptan | Droperidol aguettant | Droperiaol Arrow | Droperidol kalceks | Droperidol panpharma;
- (GB) United Kingdom: Droperiaol panpharma | Xomolix;
- (GR) Greece: Xomolix;
- (HU) Hungary: Droperiaol kalceks | Xomolix;
- (IE) Ireland: Xomolix;
- (IT) Italy: Droperidol sintetica | Droperidolo hikma | Sintodian | Xomolix;
- (LU) Luxembourg: Dehydrobenzperidol;
- (LV) Latvia: Dehydrobenzperidol;
- (NL) Netherlands: Dehydrobenzper | Dehydrobenzperidol;
- (NO) Norway: Dridol | Droperidol sintetica;
- (NZ) New Zealand: DROPERIDOL SXP;
- (PL) Poland: Xomolix;
- (PT) Portugal: Droperiaol Hikma | Droperidol kalceks | Droperidol panpharma | Xomolix;
- (QA) Qatar: Dridol | Inapsine;
- (RO) Romania: Droperiaol aguettant;
- (SA) Saudi Arabia: Doravit;
- (SE) Sweden: Dridol | Droperiaol sintetica;
- (SK) Slovakia: Xomolix;
- (VE) Venezuela, Bolivarian Republic of: Dehydrobenzperidol

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